

Encorafenib (Braftovi®) and binimetinib (Mektovi®)

Encorafenib (Braftovi[®]) and binimetinib (Mektovi[®]) are cancer drugs given in combination to treat melanoma.

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What are encorafenib (Braftovi®) and binimetinib (Mektovi®)?

Encorafenib is also called Braftovi[®]. Binimetinib is also called Mektovi[®]. Both drugs are a type of targeted therapy drug called a cancer growth inhibitor.

Encorafenib and binimetinib are given in combination to treat [melanoma](#) that:

- cannot be removed with surgery
- has spread to other parts of the body (advanced or metastatic melanoma).

Encorafenib and binimetinib treatment is only given if tests show that the cancer cells have a genetic change called a [BRAF V600 mutation](#). It is best to read this information with our general information about [targeted therapy drugs](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How encorafenib and binimetinib are given

Encorafenib is capsules that you take once a day. Binimetinib is tablets that you take twice a day, 12 hours apart.

During a course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. You will also have regular [heart tests](#) such as an ECG or echocardiogram. These blood and heart tests are to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist regularly during treatment. They will talk to you about your blood results and ask how you have been feeling.

Your course of treatment

You will keep taking this treatment for as long as it is working for you and any side effects can be managed.

Your doctor, nurse or pharmacist will give you the encorafenib capsules and binimetinib tablets to take home. They may give you capsules and tablets of different strengths. Always take them exactly as they tell you to. This is important to make sure they work as well as possible for you.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Taking encorafenib capsules and binimetinib tablets

You can take these drugs with or without food. Make sure you:

- swallow them whole with a glass of water
- do not chew, open, break or crush them
- take them at the same times every day.

If you forget to take encorafenib, you can still take your dose as long as it is not more than 12 hours late. If there is less than 12 hours until the next dose, do not take the forgotten dose. Take the next dose at the usual time.

If you forget to take binimetinib, you can still take your dose as long as it is not more than 6 hours late. If there is less than 6 hours until the next dose, do not take the forgotten dose. Take the next dose at the usual time.

Do not take a double dose of encorafenib or binimetinib to make up for a missed dose. Contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your capsules and tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused capsules and tablets. Return them to your cancer team at the hospital.

Grapefruit

You should avoid eating grapefruit and drinking grapefruit juice whilst taking encorafenib. Grapefruit can affect how the treatment works and may increase side effects.

Lactose

Binimetinib tablets contain lactose. Tell your doctor if you have a lactose intolerance.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

[If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Diarrhoea

This treatment may cause severe [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. If you have a stoma, it will be more active than usual.

Your doctor, nurse or pharmacist may give you anti-diarrhoea drugs to take at home.

If you have diarrhoea or a mild increase in stoma activity, follow their advice about:

- taking anti-diarrhoea drugs
- how much and what type of fluids to drink
- any changes to your diet that might help.

Contact the hospital straight away on the 24-hour number if any of the following things happen:

- you have diarrhoea at night
- you have uncomfortable stomach cramps
- you have diarrhoea 4 or more times in a day
- you have a moderate increase in stoma activity
- the anti-diarrhoea drugs do not work within 24 hours.

You may need to give a stool sample, or go to hospital to have fluids through a drip or antibiotics.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Feeling tired

Feeling tired is a common side effect of this treatment. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Eyesight changes

This treatment can affect your eyes and eyesight. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- sore, painful, or swollen eyes
- eyes that are sensitive to light
- changes to your eyesight.

You may need steroids or other treatments. A specialist eye doctor (ophthalmologist) may also need to check your eyes. Do not drive if your eyesight is affected.

Muscle or joint pain

This treatment can cause pain in your muscles or joints. If this happens, tell your doctor so they can give you painkillers. Tell them if the pain does not get better.

Sometimes this treatment can cause a muscle condition called rhabdomyolysis. Symptoms may include:

- muscle pain
- weakness
- dark-red or brown urine (pee).

This condition is not common and does not always cause symptoms. You will have regular blood tests to check for signs of it. Your doctor may change the dose or stop your treatment for a time to let your body recover from this side effect.

Effects on the liver and kidneys

This treatment can affect how your kidneys or liver work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys and liver are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids also helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

High temperature (fever)

This treatment can cause a high temperature. Contact the hospital on the 24-hour telephone number if you feel hot, flushed or shivery. They will check what may be causing your symptoms and will give you advice.

Blood pressure

This treatment can cause high blood pressure. You will have your blood pressure regularly checked during treatment. If needed, you will be prescribed medicines to control your blood pressure.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Bleeding

This treatment may increase your risk of bleeding. Contact the hospital straight away on the 24-hour number if you have any bleeding that you cannot explain. This includes:

- blood in your urine (pee)
- blood in your stools (poo), or black poo
- bleeding from the rectum (back passage)
- vomiting up blood
- vomit that looks like coffee grounds.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Skin changes

This treatment may cause a rash or itchy skin. Or you may notice some areas of skin become thicker than usual. Your skin in some areas may become darker. Less commonly, the skin may peel or you may get small acne-like bumps.

Your skin may become more sensitive to light than usual. If your skin feels dry, try using an unperfumed moisturising cream every day.

These changes are usually temporary and improve after treatment finishes. Always tell your doctor or nurse straight away about any [skin changes](#). They can check your skin and give you advice. They may arrange further tests or give you creams or medicines to help.

Hair loss

Your hair may get thinner or you may lose all the hair from your head. [Hair loss](#) is almost always temporary. Your hair will usually grow back after treatment ends. Your nurse can talk to you about ways to cope with hair loss.

Fluid build-up

This treatment can cause a build-up of fluid in the body. This will slowly get better after treatment ends. Contact the hospital on the 24-hour number if you:

- are gaining weight
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Dizziness

This treatment can make you feel dizzy. This is usually mild. Contact the hospital on the 24-hour telephone number if you feel dizzy or faint. If you feel dizzy, do not drive or use machinery.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Allergic reaction

Some people have an allergic reaction while having this treatment. Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

If you feel unwell or have any of these signs, contact the hospital straight away on the 24-hour number. Do not take any more of this treatment until you have checked with them.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about [coping with changes to taste](#).

Effects on the heart

This treatment can affect how the heart works. You will have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may give you tablets to manage these. They may change the type of cancer treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on the lungs

This treatment can cause changes to the lungs. Tell your doctor, nurse or pharmacist if you develop:

- a cough that does not go away
- wheezing
- breathlessness.

You should also tell them if any existing breathing problems get worse. You may have tests to check your lungs.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip
- medicines such as allopurinol tablets or [rasburicase](#) through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other types of cancer

This treatment may increase your risk of developing some other cancers, such as [squamous cell skin cancer](#). But treating the melanoma usually outweighs this risk. Your doctor or nurse will give you more information about this.

Other information

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements

- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

Reviewed: 01 September 2024 | Next review: 01 September 2026

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help